NHS A&E Performance in England, Oct 2024 to Sep 2026: Executive Summary

Nov 5, 2026

England's A&E departments saw about three in four patients within 4 hours over the two years, against a 95% standard. The shortfall is almost entirely in major emergency departments, where only six in ten patients were seen within 4 hours, and no major department met the standard.

At a glance

Measure	Figure
Patients seen within 4 hours, all A&E types (24 months)	74.6%
Major emergency departments (Type 1)	60.4%
Other departments (Types 2 and 3)	97.0%
Change in Type 1 performance, Year 1 to Year 2	59.7% to 61.0% (+1.3 points)
Major departments meeting the 95% standard	0 of 120
Range between trusts (Type 1, 24 months)	43.6% to 91.7%
Waits of over 12 hours from decision to admit	1,166,888
12-hour waits per 1,000 admissions via A&E	120.7
Major departments seeing fewer than 60% within 4 hours	61 of 120 (51%)

Year 1 is October 2024 to September 2025; Year 2 is October 2025 to September 2026.

1. National performance is about 20 points below the standard, with only modest improvement

Performance stayed between roughly 71% and 77% in every month, well short of 95%. It was lowest in December 2024 (71.1%) and highest in July 2025 (76.4%).

Month	Year 1	Year 2	Change
October	73.1%	74.2%	+1.1
November	72.2%	74.2%	+2.0
December	71.1%	73.8%	+2.7

The second winter was noticeably less severe than the first. Over the full year, though, major emergency departments improved by only 1.3 points (59.7% to 61.0%). At that rate, closing the gap to 95% would take decades, so the improvement is real but small relative to the shortfall.

2. The gap is almost entirely in major emergency departments

Minor injury units, walk-in centres and urgent treatment centres met the 95% standard every month, at 96% to 97%. Major (Type 1) departments ran at 55% to 61%, about 35 to 40 points below it.

In October 2024, Type 1 departments accounted for 609,113 of the 635,342 patients who waited more than 4 hours: 96% of all breaches. The headline all-types figure (73.1% that month) is lifted by the high-performing minor units and understates the pressure in major departments (58.1%).

The national 4-hour problem is therefore a major emergency department problem. Improvement effort and reporting are most informative when focused on Type 1 performance rather than the combined figure.

3. Winter dips happen without a matching rise in attendances

Type 1 performance fell from 58.1% in October 2024 to 55.3% in December 2024, then recovered to 61.2% by May 2025. The same pattern repeated in the second winter.

Over the same period, monthly Type 1 attendances stayed broadly flat, at around 1.4 to 1.5 million. Performance falls in winter even though roughly the same number of patients arrive.

This suggests the winter pressure comes from what happens after patients arrive, such as sicker patients needing admission and fewer free beds, rather than from patient volume. This is a hypothesis: the monthly data does not include acuity or bed occupancy, so it cannot confirm the cause.

4. Performance varies widely between trusts, and the national figure hides it

Across the 120 trusts with a major department and complete data, Type 1 performance over two years ranged from 43.6% to 91.7%. None reached 95%.

	Trust	Type 1, within 4 hours
Highest	Sheffield Children's NHS Foundation Trust	91.7%
Highest general acute	Homerton Healthcare NHS Foundation Trust	80.6%
Lowest	University Hospitals Plymouth NHS Trust	43.6%
Second lowest	The Shrewsbury and Telford Hospital NHS Trust	44.0%

Three of the top five are specialist children's trusts, which treat a different mix of patients, so the best general acute trusts are the fairer benchmark.

Change over time also varied sharply. Nationally, Type 1 performance rose by 1.3 points, but individual trusts ranged from an improvement of 18.8 points (Princess Alexandra Hospital, 54.1% to 73.0%) to a decline of 10.8 points (Plymouth, 49.0% to 38.1%). Plymouth is both the lowest performer and the largest decliner.

Grouping the 120 trusts by distance from the standard shows how widespread the pressure is:

Pressure band (Type 1, 24 months)	Trusts	Share
Meets standard (95% or more)	0	0%
Near standard (85% to 94.9%)	1	1%
Moderate pressure (75% to 84.9%)	6	5%
High pressure (60% to 74.9%)	52	43%
Severe pressure (below 60%)	61	51%

More than half of major departments saw fewer than six in ten patients within 4 hours, and 113 of 120 (94%) were below 75%. The trust-change figures above were produced before eight renamed trusts were added (see Data, method and confidence) and will be confirmed on the full 120.

5. Twelve-hour waits are concentrated and do not track 4-hour performance

Over two years, 1,166,888 patients waited more than 12 hours between the decision to admit them and their admission: about 121 for every 1,000 admitted through A&E. In the initial analysis of 112 trusts, half of all these waits occurred in just 23 trusts.

The largest number was at University Hospitals Birmingham (45,748), mainly because it is one of the largest trusts. Relative to size, the pressure was greatest elsewhere:

Trust	12-hour waits per 1,000 admissions
Croydon Health Services	534
East Cheshire	420
Medway	386
Shrewsbury and Telford	380

At these trusts, roughly two in every five admitted patients waited more than 12 hours, around three times the national rate. North Cheshire and Mersey (458 per 1,000), one of the renamed trusts now included, would also rank near the top.

Trusts with similar 4-hour performance can have very different 12-hour waits. Hampshire Hospitals (52.9% within 4 hours) recorded 11.5 waits per 1,000 admissions, while Tameside and Glossop (53.5%) recorded 335. The two measures capture different problems: 4-hour performance reflects flow through the department, while 12-hour waits reflect whether a bed is available once admission is decided. Both are needed to judge a trust's position.

Data, method and confidence

The analysis uses NHS England's Monthly A&E Attendances and Emergency Admissions statistics (MSitAE), accredited official statistics, for October 2024 to September 2026: 24 monthly files and 4,721 provider rows after cleaning.

- **Pipeline:** Python (pandas) cleaned and validated the files, Oracle SQL answered the analytical questions, and Power BI presents the results in a three-page dashboard.
- **4-hour performance** = (attendances minus attendances over 4 hours) / attendances, including booked appointments, calculated from summed counts rather than averaged percentages.
- **Trust comparisons** use only the 120 trusts with a major department and Type 1 activity in all 24 months, identified by organisation code.

Confidence in the figures is high. October 2024 performance was calculated independently in Python, SQL and Power BI, and all three gave 73.1%, against NHS England's published 73.0%. The 0.1-point difference is attributed to the source file being a later revision. Provider totals reconciled exactly to the England totals (2,360,536 attendances; 635,342 breaches), and the trust rankings match between SQL and Power BI.

Fourteen data quality issues were found and resolved or documented, including an England total row that would have doubled every national figure, and eight trusts that changed name during the period and were initially excluded from the rankings. Details are in the data quality log.

Limitations and next steps

Limitations

- **Monthly, trust-level counts only.** The data has no patient age, diagnosis, acuity or bed occupancy, so it shows where and when performance falls but cannot explain why.
- **Type 1 trust results are stricter than the official measure.** NHS England judges trusts against 95% on all A&E types; this analysis isolates major departments for a like-for-like comparison.
- **Children's trusts** are included in the rankings but are not directly comparable with general acute trusts.
- **Two years of data** show the direction of change but are too short to separate a lasting trend from year-to-year variation.
- **Some trust-level results need checking:** Croydon is missing one month of 12-hour waits (so its rate is slightly understated), and North Cheshire and Mersey's April 2026 name change may reflect a merger. Maidstone and Tunbridge Wells' recorded admissions halved from October 2025, likely a reporting change, which inflates its Year 2 12-hour wait rate (about 150 per 1,000 against about 66 on Year 1 admission levels).

Next steps

- ☐ Re-run the trust-change and 12-hour concentration queries on all 120 trusts and update Findings 4 and 5; screen every trust for step changes in recorded admissions
- ☐ Plot 4-hour performance against 12-hour waits for every trust to test how closely the two measures are related
- ☐ Add bed occupancy data to test the winter capacity hypothesis in Finding 3
- ☐ Extend the series as new months are published, using the existing cleaning pipeline

Source: NHS England, A&E Attendances and Emergency Admissions; October 2024 statistical commentary for the published 73.0% figure.